

Response and Proposed Allocation of Questions

Request for Medical Information dated 31 July 2026

Employee: Cory Lea Shepherd, AO3, Switchboard Services, Logan Hospital

Reference: MSH-INJ-5795 · CLM-317073

Request issued by: Human Resources, Logan and Beaudesert Health Service — 31 July 2026

Response date: 3 August 2026

IN SUMMARY

1. I am engaging with this process and will obtain and provide the information sought.
2. Four of the nine questions are not medical questions at all. Questions 2, 3, 6 and 9 ask my doctor to answer matters which the Health Service's own policy and the Work Health and Safety Act 2011 place with the Health Service. I ask that it answer those itself, after which the properly medical part of each can be put to my practitioners.
3. Of the remainder, Questions 1(c), 1(d), 4, 5 and 8 are for my general practitioner, and Questions 1(a), 1(d), 7 and 8 require or are better answered by my treating psychiatrist. I will obtain all of them. Dr Ma's understanding of my psychiatric condition derives from my psychiatrist's correspondence to him, which is why several questions are better put to the specialist directly — and is the answer to Question 1(c).
4. Question 1(a) I answer openly in this document. Question 1(b) alone I ask be withdrawn: it asks a general practitioner for a psychiatric causal formulation, and it goes to a matter in issue in WC/2024/227.
5. I ask to be returned to my roster now. The restriction in issue asks that complaints be redirected rather than absorbed — which is the process the Health Service says it already operates, and which the Role Description does not require of me in any event.
6. I ask for an extension of the seven days, and explain at Part 7 why that period cannot be met on the Health Service's own question set.
7. I ask who the delegate is, and propose at Part 6.1 how the conflict can be managed: the medical information held exclusively by Injury Management, the decision made outside Corporate Services, and implementation retained within it.
8. My proposal for returning to work, and the confirmation of my substantive position, are in the accompanying Proposal for Return to Work. Part 9 lists what I ask the Health Service to confirm.

PART 1 — THE ORIGIN OF THE REQUEST, AND THE TEST APPLIED

1.1 THE ORIGIN AND STATED PURPOSE OF THE EMPLOYEE CAPABILITIES CHECKLIST

The letter of 31 July 2026 records the origin of the checklist in the Health Service's own words:

"your recently completed Employee Capabilities Checklist (ECC) (Attachment 1) completed by Dr Day Hong Ma on the 3 July 2026 which was provided to us to facilitate a Return to Work Plan."

That is correct, and the sequence was as follows.

- (a) On 5 May 2026 the Manager, Switchboard Services asked me to arrange for the Employee Capabilities Checklist to be completed, so that the Health Service could "provide the most appropriate support in the workplace" and "discuss the next steps to ensure you are supported and working safely". No condition was attached to my attendance.
- (b) On 21 May 2026 I advised that an appointment was booked for 5 June 2026. In the same email I arranged a swap for a shift I was rostered on the following day. I remained rostered and at work with the checklist outstanding, and no suggestion was made that I could not attend.
- (c) On 26 June 2026 the position changed. I was advised that "until we receive appropriate medical clearance, including a completed Employee Capacity Certificate (ECC), we are unable to facilitate your return to work", by reference to section 19 of the Work Health and Safety Act 2011, and that without the documentation I "will be unable to return to work".
- (d) That position was repeated on 2 July 2026, with a leave-type deadline of 2:00pm on Friday 3 July 2026.
- (e) I obtained the checklist and provided it on 3 July 2026 at 2:32pm. It certified me fit to work with restrictions.

(f) At 3:18pm on 3 July 2026 I asked the Health Service to confirm whether I was to attend, or otherwise to advise my next returning shift. That request has not been answered. My rostered shifts were charged to accrued leave, then to sick leave without pay, and from 13 July 2026 to income protection without pay. I have not worked since.

The checklist was therefore required by the Health Service, for a purpose the Health Service has itself stated — to facilitate a return to work — and, once provided, became the basis on which work was refused. It did not certify me unfit. It certified me fit with restrictions.

I record this because it bears directly on the present request. The Health Service now seeks further information about a document it required me to obtain in order to work, and which certified that I could.

I am willing to provide clarification of that document. I ask that the clarification be directed to identifying the adjustments which allow me to work — the purpose for which the document was obtained, the purpose stated in the letter of 31 July 2026, and the purpose contemplated by Human Resources Policy G03.

1.2 THE TEST APPLIED

The letter to Dr Ma states the purpose of the request:

"to identify and implement any appropriate workplace controls or reasonable adjustments to support a safe and sustainable return to his substantive position."

Human Resources Policy G03 (QH-POL-210), which the Health Service has cited, sets the limit at clause 3:

"Reasonable adjustment may need to be supported by medical or rehabilitation information, where necessary, to either identify required workplace adjustments and/or assess the suitability of proposed adjustments."

I have applied that test to each question: is this information necessary to identify a required adjustment, or to assess the suitability of a proposed one — and is the person being asked the right person to answer it?

Applying that test, the nine questions divide four ways. I am not declining to provide information. I am proposing that each question be directed to whoever is able to answer it.

1.3 THE CHECKLIST ALREADY ANSWERS MOST OF WHAT IS NOW ASKED

The Employee Capabilities Checklist is Attachment 1 to the Health Service's own letter. Read in full, it already answers, in substance, most of the nine questions.

Q1(d) foreseeable risk and necessary controls — answered at Additional Comments 3 and 4, which set out the recommended work pattern with clinical reasoning for each shift type, and five specific workplace modifications.

Q4 restrictions, clinical basis in functional terms, duration and review — answered at Additional Comments 2: "Episodic, with symptom exacerbation linked to stressor exposure. The restrictions below are expected to remain necessary for at least the next 3-6 months, subject to treatment response and 8-weekly review." The review date is given on the first page as 28 August 2026.

Q5 what "complaint handling" encompasses — answered: "complaints received at the Switchboard are to be logged and redirected to management/the dedicated complaints area, not actioned, resolved or absorbed by Mr Shepherd."

Q6 the requirements of the role — answered: "Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below."

Q7 working memory in functional terms — addressed under Memorising information ("Working memory affected under stress") and at Additional Comments 1 ("reduced concentration and working memory under sustained high-demand load"), with the roster controls which manage it.

Q8 tasks and environments which exacerbate the condition — answered at Additional Comments 1 and in the foreseeable-stressors response.

Q2 capacity to follow a direction — the checklist records "Understanding instructions: Normal" and "Carrying out instructions accurately: Normal."

I do not raise this to refuse further clarification. I have set out at Part 3 what my practitioners will provide. I raise it because I have been excluded from the workplace for five weeks while information the Health Service has held since 3 July 2026 is sought again.

1.4 THREE MATTERS ON THE FACE OF THE CHECKLIST

- (a) **It records a continuation, not a return.** The first page states: "Fit to commence the above from: 3 July 2026 (continuation of existing arrangement)."
- (b) **It records twelve months of sustained work.** At Additional Comments 3: "This reduced pattern of approximately six shifts per fortnight is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration, and maintaining rather than increasing it best supports recovery while minimising risk of exacerbation."
- (c) **The restriction in issue asks for nothing the Health Service does not already do.** The checklist asks that complaints be "logged and redirected to management/the dedicated complaints area." The letter of 31 July states that the established process is "immediate escalation and management by the Health Service Client Liaison Officer and/or Manager, Switchboard Services." Those are the same arrangement. The Role Description at Attachment 2 contains no complaint-handling responsibility.

I record also that the checklist recommends, at Additional Comments 4(c), "return-to-work coordination and correspondence through the Injury Management team, Human Resources." That is the substance of what I propose in the accompanying Proposal for Return to Work.

PART 2 — PROPOSED ALLOCATION

Q	SUBJECT	PROPOSED RESPONDENT
1(a)	Date of first diagnosis	Answered openly; clinical context from the psychiatrist
1(b)	Clinical basis for the causal link	Withdrawal requested — a psychiatrist question, and in issue in the appeal
1(c)	Self-report v clinical assessment; other reports held	Consented to — Dr Ma. Answer: the specialist reports he holds, not self-report
1(d)	Foreseeable risk; controls medically necessary	Psychiatrist; Dr Ma from the specialist documentation he holds
2	Following directions; performance and conduct discussions	Health Service — threshold question
3	Fitness under the existing reporting arrangements	Health Service — risk assessment, then treating psychiatrist
4	Restrictions, adjustments, duration and review date	Dr Ma (GP)
5	Meaning of "complaint handling"	Health Service — already answered, then Dr Ma
6	Requirements of the role without modification	Health Service — identify genuine occupational requirements, then GP
7	"Working memory affected under stress"	Treating psychiatrist
8	Tasks and environments that exacerbate the condition	Psychiatrist; Dr Ma from the specialist documentation he holds
9	If unable to accommodate, whether safe return is possible	Health Service — onus is the employer's

Of the nine questions, four are not medical questions. They are questions the Health Service must answer itself before any clinician can usefully be asked anything.

This allocation applies to whatever question set the Health Service confirms. If questions are withdrawn, amended or added, I will allocate the confirmed set on the same basis — each medical question to whichever of my practitioners is the right person to answer it, and the questions at Part 4 to the Health Service.

PART 3 — QUESTIONS FOR MY PRACTITIONERS

Dr Ma completed the Checklist and can answer within general practice and from his own file. My psychiatric condition is managed by my treating psychiatrist, and Dr Ma's understanding of it derives from that specialist's correspondence to him — which is also, in substance, the answer to Question 1(c): his view of the identified stressors rests on the specialist reports he holds, not on my self-report alone.

3.1 FOR DR MA — I will obtain answers to these

Q1(d) foreseeable risk and the controls medically necessary — from the specialist documentation he holds; because the risk is psychiatric, I will also put it to my psychiatrist.

Q4 restrictions, clinical basis in functional terms, duration and review date.

Q5 what "complaint handling" encompasses as a clinical restriction — the operational meaning is dealt with at Part 4.3.

Q8 tasks, situations or environments which may exacerbate the condition — likewise also put to my psychiatrist.

Q1(c) I consent to this. Dr Ma can confirm what he holds and has relied on — his own file — and I ask only that the question not be read as an inquiry into material held by others.

The Checklist form states that "a separate medical certificate is also required". One was not provided on 3 July 2026; I will obtain it when I rebook with Dr Ma, or sooner if required.

3.2 FOR A TREATING PSYCHIATRIST — I am willing to obtain this

Q1(a) I answer this openly: the diagnosis was made on 24 October 2024. What makes a date meaningful is the clinical context, which is my psychiatrist's to give; and the question as framed asks a general practitioner to certify that no other diagnosis exists, which is beyond his scope. I ask that it be put in psychiatric terms.

Q1(d), Q8 as above — a specialist view of a psychiatric risk and its exacerbating factors.

Q7 what "working memory is affected under stress" means in functional terms — a functional cognitive opinion is a psychiatrist's, not a general practitioner's.

Please confirm which questions the Health Service presses. I will then instruct my psychiatrist, with the invoice directed to you consistent with the Health Service's confirmation that it will meet the reasonable costs. The opinion will be directed to current capacity, functional restrictions and recommended adjustments — the stated purpose of this inquiry — and not to the cause of my injury, which is in issue in WC/2024/227 and is not necessary for that purpose.

The sequence bears on the timeframe: I cannot instruct either practitioner until the question set is settled, and none of that can occur within seven calendar days. The only step which cannot presently begin is the one which depends on the Health Service. I note that a request to my own treating practitioners rests on a different footing from an independent medical examination under chapter 3 part 8 division 5 of the Public Sector Act 2022, which has not been invoked.

PART 4 — QUESTIONS FOR THE HEALTH SERVICE

These four questions ask a doctor to answer matters which the Health Service's own policy and the Work Health and Safety Act 2011 place with the Health Service. I ask that the Health Service answer them, after which the properly medical component of each can be put to my practitioners.

4.1 QUESTION 2 — is there a performance or conduct process?

The question asks whether I can "follow a reasonable and lawful direction issued by [my] supervisor" and "participate in discussions in relation to [my] workplace performance and/or conduct."

These are not questions about capacity to perform the genuine occupational requirements of my role. They concern readiness for a management process. Either such a process is on foot concerning me — in which case I am entitled to be told of it, given particulars, and afforded procedural fairness — or there is not, in which case the questions do not serve the stated purpose of this inquiry.

→ **FOR THE HEALTH SERVICE: please confirm whether any performance or conduct process concerning me is on foot or contemplated, and if so provide particulars.**

→ **THEN, IF APPROPRIATE, FOR MY PRACTITIONER: "Does the employee have capacity to participate in ordinary workplace meetings and supervision, and are any supports required?"**

The premise of Question 2 is separately incorrect and is addressed at Part 5.

4.2 QUESTION 3 — is the workplace safe?

The question asks whether I am "medically fit to return to their substantive role under the existing reporting arrangements, including working with and reporting to their current line manager." That asks a treating doctor to certify that a workplace arrangement is safe. Under section 19 of the Work Health and Safety Act 2011 that duty rests with the

Health Service; under the Managing the risk of psychosocial hazards at work Code of Practice 2022 a psychosocial hazard is a feature of the work, identified and controlled by the PCBU; and G03 clause 1 requires proper consideration of that Code. A doctor can advise on me; a doctor cannot assess the Health Service's reporting arrangements.

The letter of 7 July 2026 records that the Health Service "does not have sufficient information to understand the nature of these matters or assess whether they present any psychosocial hazards or risks". That gap is real, and it is closed by the Health Service consulting and assessing — sections 47 to 49 require consultation with workers directly affected, and none has occurred since 26 June 2026. For completeness, the matters concern how work in the unit is organised and decided: how patient safety concerns raised at the switchboard are escalated; how changes to work and roster have been introduced; fatigue and recovery between shifts; and staff input into decisions affecting the work. They are workplace matters, which is why the assessment belongs with the Health Service.

→ **FOR THE HEALTH SERVICE: a risk assessment of the Switchboard work environment addressing the psychosocial hazards identified. Clause 7.2.1 of the certified agreement provides the mechanism.**

→ **THEN FOR MY PRACTITIONER: "Are any adjustments recommended in relation to supervision arrangements, and if so, what are they in functional terms?"**

4.3 QUESTION 6 — what are the genuine occupational requirements?

The question asks whether I can "fulfill the full inherent requirements of [my] Role ... without restrictions or modifications to duties."

This applies the wrong test. G03 states its purpose as assisting employees "to meet the genuine occupational requirements of their role by applying principles of reasonable adjustment," and at clause 3 provides that adjustments "should support the employee to undertake the genuine occupational requirements of their role, without risk to their own health and safety or that of others in the workplace." Section 34 of the Anti-Discrimination Act 1991 is to the same effect. Neither asks whether an employee can perform a role without adjustment.

A clinician also cannot determine which duties are genuine occupational requirements. That is the employer's determination. The Health Service has itself already made part of it: its letter states that "established process for client complaints received by Switchboard employees is for immediate escalation and management by the Health Service Client Liaison Officer and/or Manager, Switchboard Services," and the Role Description at Attachment 2 contains no complaint-handling responsibility.

→ **FOR THE HEALTH SERVICE: identify which duties of the role it says are genuine occupational requirements, having regard to the Role Description at Attachment 2.**

→ **THEN FOR MY PRACTITIONER: "Can the employee perform the genuine occupational requirements of the role with the adjustments you have recommended?"**

4.4 QUESTION 9 — can the adjustments be accommodated?

The question asks: "If we are not able to accommodate the restrictions you have recommended, is Mr Shepherd able to safely return to the workplace?"

This presupposes an inability to accommodate before any accommodation has been assessed. G03 clause 2 provides that unjustifiable hardship "is tested against the whole organisation, not a division or unit within the organisation," and that "the onus is on Queensland Health, as the employer, to prove an adjustment is unreasonable, not on the person to prove that it is reasonable."

The question therefore asks my doctor to answer the final step first, on a hypothetical the Health Service has constructed and not yet tested, and on which the onus is not mine.

→ **FOR THE HEALTH SERVICE: assess the recommended adjustments against the whole organisation. If it considers any cannot be accommodated, identify which and its reasons.**

→ **THEN, IF NECESSARY, the question can be put to my practitioner properly.**

PART 5 — THE QUESTION I ASK BE WITHDRAWN, AND A CORRECTION OF FACT

5.1 WITHDRAWAL REQUESTED

Q1(b) "What is the clinical basis for identifying these specific workplace stressors as contributing to the exacerbation of Mr Shepherd's condition." This is not a question a general practitioner can properly answer.

The clinical basis for a psychiatric causal formulation is a matter for a psychiatrist. It is also a question I ask be

withdrawn. For the purpose of identifying controls, what matters is that the stressors exacerbate my condition and what controls address them — both of which are obtained by Questions 1(d) and 8. The clinical reasoning as to causation goes to a matter in issue in WC/2024/227 and is not necessary for the stated purpose of this inquiry.

I record that section 124(1) of the Anti-Discrimination Act 1991 provides that a person "must not ask another person, either orally or in writing, to supply information on which unlawful discrimination might be based," and that under section 124(3) the defence requires the respondent to prove "that the information was reasonably required for a purpose that did not involve discrimination." Impairment is a protected attribute. I raise this not as a complaint but so that the Health Service can satisfy itself that each question it presses meets that standard.

5.2 CORRECTION OF FACT

Question 2 is premised on the statement that "the Health Service is not aware of any concerns being raised for appropriate management."

That statement is incorrect. Concerns have been raised in writing on multiple occasions, including a public interest disclosure (PID 24-ESU-1130); a written patient safety report; correspondence to Human Resources over a period exceeding two years concerning fatigue, rostering and the management of directory information; a written complaint to the A/Director, Human Resources Business Partnering dated 3 October 2025; and the matters in WC/2024/227, in which the Health Service filed an objection dated 5 June 2026 under the signature of the Health Service Chief Executive.

I do not ask the Health Service to agree with any of those concerns. I ask only that my treating practitioner not be asked to answer a question which assumes they were never made. I request that the correction be provided to Dr Ma in writing before he is asked to answer any question resting on that premise.

PART 6 — DELEGATION AND CONFLICT OF INTEREST

This is not a personal complaint about any individual. I make no allegation and I ask for no finding about anyone. My concern is with how the process is being run — who is deciding, on what material, and under what instrument — and with the support and adjustment that ordinarily apply when an employee is certified fit with restrictions. G03 clause 1.1 provides for temporary adjustments so an employee may "continue work during their recovery"; the Workplace rehabilitation standard requires suitable duties to be actively sought and the worker's connection with the workplace maintained, for "either a work or non-work-related injury/illness". I obtained the checklist the Health Service required for that purpose. It certified me fit with restrictions. What followed was exclusion rather than adjustment.

Due process is the other half: being told what power is being used and who decides; a fair opportunity to respond before a serious employment consequence; consultation on the matters in issue; deadlines that are not one-sided; and decisions made by an officer independent of the matters they concern — what the Managing the risk of psychosocial hazards at work Code of Practice 2022 calls organisational justice. I do not say any tribunal has found a denial of fair process; I say that process has not been applied, and I ask that it be.

I asked on 28 July 2026 who is making decisions about me and whether any conflict of interest has been identified. I have not received a response. Both letters of 31 July 2026 are written in the first person of the decision-maker and signed by Mr Hughes, Director, Corporate Services; question 3 asks my doctor about the reporting arrangements within that same directorate, and the Manager who gave the advice of 2 July 2026 reports within it. The officer determining my return therefore sits within the arrangements in issue. I ask the Health Service to confirm: (a) the source and instrument of the delegation; (b) whether any conflict has been identified, declared and managed, and how; and (c) whether the decisions of 2 and 3 July 2026 were made with the knowledge or approval of the officer now determining my return.

6.1 A PROPOSED MANAGEMENT OF THE CONFLICT

I propose the ordinary separation of functions: the clinical material held exclusively by the Injury Management team on a medical-in-confidence basis — which is what the Checklist itself recommends at Additional Comments 4(c); any decision concerning my capacity or return made by an officer outside Corporate Services; and implementation — rostering, and putting the adjustments in place — retained within Corporate Services. The officer steps out of the decision, not out of the workplace. I record, because it bears on the assessment, that on 3 October 2025 I made a formal written request for review to the A/Director, HR Business Partnering concerning a process initiated by the officer who has signed the letters of 31 July 2026; I do not reopen it here. Code of Conduct clause 1.2 provides that a conflict is not wrongdoing in itself — failing to disclose and manage one is likely to be.

I have written separately to the Chief Executive today about the conflict and the handling of my personal information, because section 89 of the Public Sector Act 2022 makes the chief executive the person to whom a conflict is disclosed. That letter deals with the matter in full.

PART 7 — TIME

7.1 DEADLINES IN THIS MATTER HAVE RUN IN ONE DIRECTION

I met the deadline of 2:00pm on 3 July 2026; the checklist was provided at 2:32pm and certified me fit with restrictions. At 3:18pm I asked whether I should attend. Five weeks later that has not been answered, no instrument authorising my exclusion has been identified although I have asked three times, no date has been given by which I might return or any decision would be made, and on 30 July 2026 I was told the matter was "progressing for delegate approval" and that the Health Service was "unable to guarantee" my return.

On 31 July 2026 the Director, Corporate Services set me seven calendar days, with a consequence: a decision regarding my ability to perform my role on the available information. The letter does not say what that decision is. I ask that it be identified — what decision, and under what instrument. If it is a decision that I return to work, I welcome it and no consequence arises. If it is a decision of another kind, I am entitled to know now, and to the procedural fairness that attaches to it. This is the second occasion an adverse position has been foreshadowed without an instrument: on 15 July my return was said to "remain at the discretion of the Employer".

I do not object to a deadline. I note only that I have met every deadline set for me, and in five weeks without income I have been given none in return. I ask for the extension below, and for a date by which the Health Service will tell me whether I am returning to work.

7.2 THE EXTENSION

The letter allows seven days, expiring 7 August 2026, and expressly contemplates a reasonable extension. I request one. The letter arrived at 11:43am on 31 July, two hours before a consultation with Dr Ma of which the Health Service had notice; because the question set is in issue I cancelled it rather than waste a cost, and will rebook once the set is settled. I propose two parts: (a) Dr Ma's questions — 21 days from the Health Service confirming the final question set; (b) specialist questions — on a timeframe set once it confirms which it presses and that it will meet the cost. I do not seek delay; the exclusion is costing me my income, and it cannot be reasonable to maintain an unpaid exclusion for a period created by the Health Service's own requirements.

PART 8 — DISCLOSURE AND COSTS

I adopt option 1 and will obtain the information from my practitioners and provide it. I do not provide an authority under option 2. G03 clause 3 acknowledges "the right of the person... to choose how disclosure of information occurs", and that information need only be shared where genuine occupational requirements or safety are at risk and reasonable adjustment is required. My election exercises a right the policy expressly recognises.

As to handling, I ask that any medical information I provide be: (a) held by the Injury Management Consultant on a medical-in-confidence basis; (b) not provided to my line manager or Switchboard Services — a line manager requires the adjustments, not the clinical information behind them; and (c) used only for the stated purpose. Please confirm that, and identify any person outside the Health Service to whom it is proposed to be provided. My separate letter to the Chief Executive of today's date deals with information handling in full.

As to costs, the letter of 31 July 2026 confirms that MSH will meet the reasonable costs of the reports and directs invoices to you. I have proceeded on that basis; both practitioners will invoice you directly, quoting MSH-INJ-5795. The consultation of 3 July 2026 (\$150.00, invoice 574370, paid 10 July 2026) is an earlier cost whose reimbursement I sought on 30 July 2026 and which remains outstanding.

PART 9 — RESPONSE REQUESTED

I ask for a written response confirming:

1. whether I am returned to my roster, and if not, the instrument relied on to maintain my exclusion and whether the period will be treated as paid special leave with my leave re-credited;
2. the delegate, the source of the delegation, how any conflict of interest has been identified and managed, and whether the separation of functions proposed at Part 6.1 is agreed — the medical information held exclusively by Injury Management, the decision made outside Corporate Services, and implementation retained within it;

3. whether any performance or conduct process concerning me is on foot or contemplated;
4. which duties of my role the Health Service says are genuine occupational requirements;
5. whether the Health Service will conduct the workplace risk assessment;
6. which questions it presses and which it withdraws;
7. that the information will be used only for the stated purpose, who outside the Health Service it is proposed to be provided to, and that it will be handled within the Health Service as set out at Part 8; and
8. what decision is meant by "a decision regarding your ability to perform your role", and the instrument under which it would be made;
9. a date by which the Health Service will tell me whether I am returning to work, as sought at Part 7.1; and
10. that my substantive position continues to be held for me, as set out in the accompanying Proposal for Return to Work; and
11. its response to the accompanying Proposal for Return to Work, which is made as a request under clause 10.3.2 of the certified agreement and to which clause 10.3.6 applies.

I confirm again that I am engaging with this process and want to return to work.

Cory Lea Shepherd AO3, Switchboard Services, Logan Hospital 0417 400 227 · coryshepherd1@hotmail.com